

YOUR RESULTS, IN PLAIN LANGUAGE

You've tried a few of these. Some did nothing. Some made things worse.

There may be a reason, and it may have nothing to do with you as a person. It might be how your body handles the medicine itself. What follows won't tell you what to take. It is here to help you and your doctor ask better questions.

Read here, on your device. Only a short derived result ever left it.

01 Your story, first

Before anything technical, this is what you told us you had tried, and what happened each time.

- 2019 **fluoxetine**
it helped for a while
"worked for about a year, then it faded. back on it now"
- 2021 **paroxetine**
I felt worse
"two weeks. couldn't stand it"
- 2022 **escitalopram**
nothing changed
"stayed on it six months to be sure"
- 2023 **mirtazapine**
I stopped
"slept 11 hours and gained weight"
- 2024 **sertraline**
helped a little

That's a lot to have been through.

02 Then the biology

Your raw genome is never uploaded.

It is read on your device. Only a short derived result, which genes run fast or slow, ever leaves it. The file itself stays yours.

- ✓ File read
- ✓ Relevant genes found
- ✓ Variants translated
- ✓ Checked against current meds

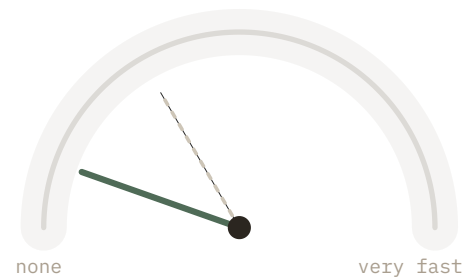
03 How you metabolize

CYP2D6

The enzyme your liver uses for paroxetine, fluoxetine, venlafaxine and most tricyclics.

Right now, barely at all.

Your copies run below typical speed. The fluoxetine you take blocks what activity is left, so functionally this enzyme is close to off this week.



The dotted line is your DNA alone. The solid needle is your body this week, with fluoxetine on board. A report that stopped at your genes would have placed you in the wrong spot.

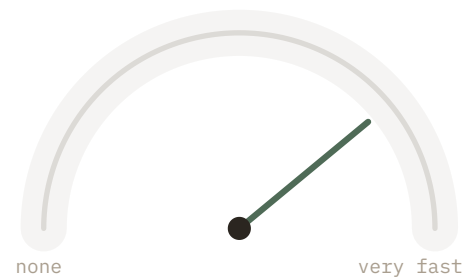
How sure are we? High. Both copies of this gene were readable in your file.

CYP2C19

The enzyme that handles escitalopram, citalopram, and part of sertraline.

You clear these quickly.

A standard dose may never reach the level it was designed to hold in you.



How sure are we? Moderate. Reference data is thinner for your ancestry here, so the exact speed is less certain. The direction, faster than typical, is solid.

04 Your story again, with what we found

● 2019 fluoxetine

helped for a while, then faded

Complicated. It slows its own exit, so levels drift for months. An early sweet spot can quietly become too much.

2021 **paroxetine**

two weeks, felt awful

Cleared very slowly in you. A standard dose likely built up faster than intended. Feeling awful by week two fits that.

2022 **escitalopram**

six months, nothing changed

Cleared faster than typical in you. A standard dose may never have reached the level it needed. Six months of waiting on too little.

2023 **mirtazapine**

stopped it yourself

We can't explain this one from your genetics. The sedation and weight gain were likely the drug being itself. Something to name, not decode.

2024 **sertraline**

helped a little

This one sits closest to your profile. The small signal you felt may have been real. Worth noting.

These medications may not have failed
because of you.

Two genes explain three of the five things you tried. One was dosed for a body that clears it faster than yours. Another for one that clears it slower.

05 What might be different

Not advice. A map of what your body does with the usual options, to bring to the person who prescribes.

● STRAIGHTFORWARD FOR YOU

sertraline already showed a small signal

Only partly handled by your fast CYP2C19, and barely by CYP2D6. Levels in you should sit near what the guidelines assume. The 2024 signal supports that.

CPIC, SSRIs and CYP2C19, 2023 update

bupropion leans on neither enzyme

Metabolized by CYP2B6, which looks typical in you. One caution to raise: it inhibits CYP2D6, which matters if anything 2D6-dependent stays in the picture.

DPWG, bupropion, 2023

○ WOULD NEED A DIFFERENT STARTING POINT

nortriptyline workable, started low

A tricyclic your slow CYP2D6 clears poorly. Guidelines suggest a reduced starting dose and a blood level check rather than ruling it out.

CPIC, TCAs and CYP2D6, 2016

venlafaxine same slow enzyme as paroxetine

Runs through CYP2D6. With yours effectively off, the parent drug accumulates. Possible, but it would need a gentler start and a doctor watching early side effects.

DPWG, venlafaxine, 2023

○ MAY EXPLAIN PAST TROUBLE

paroxetine the two weeks in 2021

Slow clearance plus a drug that inhibits its own enzyme. In you that compounds. The guideline suggests dose reduction or an alternative for poor metabolizers.

CPIC, SSRIs and CYP2D6, 2023 update

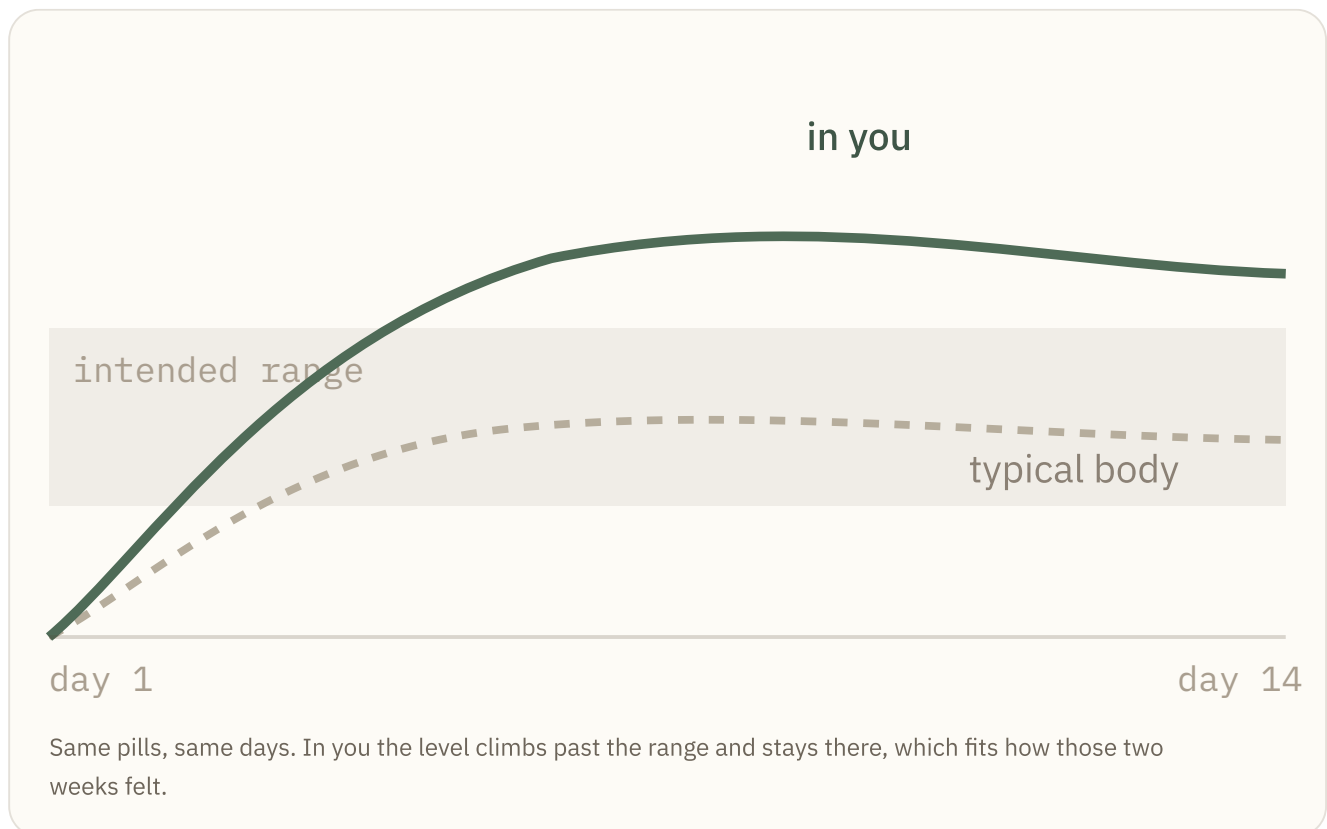
escitalopram the flat six months in 2022

Rapid CYP2C19 lowers exposure at standard doses. Guidelines note the risk of under-treatment and suggest considering this in non-responders.

CPIC, SSRIs and CYP2C19, 2023 update

What a standard dose of paroxetine did inside you

A model, not a measurement. The band is where the drug is meant to sit.



Wondering "what if"

Everything above re-derives when the picture changes.

what if I add bupropion?

Bupropion blocks CYP2D6 too. Stacked on your fluoxetine, that enzyme would sit even closer to off. Not a no, but a thing your doctor should choose on purpose.

what if I stop the fluoxetine?

Over four to six weeks your CYP2D6 would drift back toward its genetic middle. Anything it handles would clear a little faster again. Timing a switch matters here.

what if cost matters?

Sertraline and bupropion are both decades-old generics, usually a few dollars a month. Nothing in your profile points only at expensive options.

what about pregnancy?

That changes the weighing, not the genetics. Sertraline is among the better studied options in pregnancy. This one belongs with your doctor early, not after.

06 If sertraline is the one you land on

DO NOT SKIP THIS ONE

Never combine it with an MAOI, or start within two weeks of stopping one. The interaction is dangerous. Your pharmacist checks this too, but it belongs at the top.

MORNING	With breakfast This one is activating for many people. Evening doses tend to disturb sleep, so mornings are the usual habit.
FOOD	No restrictions Take it with or without food. With a little food if your stomach is sensitive.
AVOID	Alcohol, especially early on The drowsiness stacks, and it muddies your read on whether the medicine is working.
WATCH	First two weeks: stomach, sleep, jitters Common, and they usually settle. If they don't ease by week three, that's worth a call, not toughing out.

FDA label, sertraline. Maudsley Prescribing Guidelines, 14th ed.

07 For your doctor

You can't prescribe. They can. This is a one-page summary in their language, skimmable in under a minute.

PHARMACOGENOMIC SUMMARY · PATIENT-SHARED

18 JUL 2026

CYP2D6

*1/*4 · Intermediate

Phenoconverted to poor by current fluoxetine (strong 2D6 inhibitor).

CYP2C19

*1/*17 · Rapid

Reduced exposure to 2C19 substrates at standard dosing likely.

HISTORY (PATIENT-REPORTED)

fluoxetine '19 partial, faded · paroxetine '21 intolerant, d/c wk 2 · escitalopram '22 non-response 6 mo · mirtazapine '23 d/c (sedation, weight) · sertraline '24 partial response · current: fluoxetine 20 mg

GUIDELINE-CONCORDANT CONSIDERATIONS

CYP2C19 RM: escitalopram/citalopram standard dosing may under-expose (CPIC 2023, moderate). CYP2D6 PM (functional): avoid or reduce paroxetine, TCAs at standard dose; if TCA, consider level monitoring (CPIC 2016). Sertraline acceptable across both phenotypes; consider 2D6-independent options (e.g. bupropion augmentation) pending clinical judgment.

Array-based; CNV not resolved. Decision support only, not a prescription. CPIC/DPWG cited.

Questions worth asking, from your results

- "Does my CYP2C19 result explain why escitalopram never did anything at 10 milligrams?"
- "Given my CYP2D6 result, should anything new be started lower than the usual dose?"
- "The fluoxetine I'm on is slowing that enzyme further. Does that change the plan or the timing?"
- "Is staying with sertraline and adjusting it a reasonable option, given it helped a little?"

08 The weeks after

The waiting is the hardest part. It has a shape. This is roughly how the first weeks tend to go once you start something new.

- WEEKS 1 TO 2
Side effects usually arrive before benefits. Stomach, sleep, a wired feeling. This is the noisy part.
- WEEK 3 **YOU ARE HERE**
Usually too early for mood to move. Sleep or appetite sometimes shifts first, and counts as signal.
- WEEKS 4 TO 6
Where the first real change tends to show, when it's coming. Often other people notice before you do.
- WEEKS 8 TO 12
The honest verdict window. Enough time at a working dose to call it, either way, with your doctor.

Week three. How's it going?

A few taps. Never a score out of ten. Whatever you answer, it is met against what week three usually feels like.

A LITTLE LIGHTER → EARLY, AND GOOD

Earlier than the pattern predicts, which happens. Take it. Week four is when we'd expect it to hold.

ROUGHER → WORTH A CALL

Side effects can peak around now, but rougher mood in week three isn't the pattern we predicted. Not an emergency read, but worth raising with your doctor this week.

ABOUT THE SAME → ON PATTERN

This is what week three usually feels like. The change we're watching for tends to show around week four. Nothing here says it isn't working.

HARD TO TELL → ALSO AN ANSWER

Honest and common at week three. If it's still hard to tell at week six, that itself is information your doctor can use.

You don't have to carry the calendar in your head.